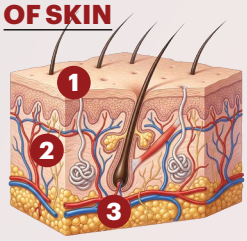
KNOW YOUR SKIN

Skin is more than a protective covering: the body's largest organ, it regulates temperature, senses touch and pain, and interacts with the immune system. Its appearance and behaviour can also offer clues to underlying health problems

THE THREE LAYERS OF SKIN



1. EPIDERMIS

- The skin's first barrier against the outside world
- Contains melanin-producing cells that give skin its colour and help protect it from ultraviolet light

2. DERMIS

- The middle layer, it contains nerves, blood vessels, sweat and oil glands, and hair follicles
- Provides strength and elasticity; helps sense heat, cold, pressure and pain

3. HYPODERMIS

- The deepest layer made up largely of fat and connective tissue
- Cushions the body and stores energy

WHY THE SKIN ITCHES

- **A warning signal:** Skin nerves detect irritants and trigger itching, prompting us to scratch and remove the perceived threat before it causes further harm
- **The cycle takes over:** A damaged skin barrier loses moisture and becomes more vulnerable to irritants and allergens. Dryness, itching, scratching and inflammation can then reinforce one another
- **The wider network:** Skin interacts with the immune system, nerves and hormones, all of which can influence how itching develops and persists

WHAT THE SKIN CHANGES SIGNIFY

- **Dark, velvety patches around the neck or armpits:** May indicate insulin resistance, prompting checks for prediabetes or diabetes
- **Slow-healing wounds or repeated skin infections:** May signal poorly controlled blood sugar
- **Persistent itching without much of a rash:** Can be associated with chronic kidney disease or liver and bile-related problems, which cause itching from within
- **Very dry, coarse skin:** Can sometimes indicate an underactive thyroid
- **Blue lips or fingers, or unusually pale, cold skin:** Can point to inadequate oxygenation or poor blood circulation
- **Yellow skin or eyes:** May indicate jaundice; yellowish fatty deposits around the eyelids or tendons can be associated with high cholesterol

reaction, particularly in sensitive skin. A 2024 Indian study published in the *Indian Journal of Dermatology, Venereology and Leprology* examined 80 patients with cosmetic dermatitis. Of these, 57 had positive patch or photopatch test results, which identify specific substances in cosmetics that may cause an allergic reaction with 50 reactions found to be relevant to cosmetic use. The researchers also identified several previously unrecognised photoallergens—ingredients that can trigger a reaction when combined with exposure to light.

Social media beauty trends add another layer to the problem. Blindly following an influencer's skincare routine can prove costly for the skin. "Skin is different from person to person," says Dr Hemant Sharma, senior consultant in dermatology at BLK-Max Hospital at Pusa Road in Delhi. "A routine that works beautifully for an influencer may irritate someone with sensitive skin, eczema or an allergy. Too many acids, retinoids, scrubs or active ingredients can damage the skin." Everyday habits can also undermine the skin's protective barrier. Frequent bathing, harsh soaps and prolonged exposure to dry, air-conditioned environments strip the skin of moisture, says Sanke. Cosmetics, fragrances and detergents can trigger contact allergies, as can nickel in jewellery and latex. The result is that itching may not stem from a single allergen but from the cumulative effects of sweat, friction, pollution and repeated exposure to products.

The face, scalp and eyelids are particularly vulnerable to cosmetic-related reactions. A 2024 Indian study published in the medical journal *Cureus*, involving 65 patients with cosmetic dermatitis, found that the face was affected in 81.5 per cent of cases, the scalp in 53.8 per cent and the eyelids in 50.8 per cent. Hair dye was the most commonly used cosmetic among the patients, followed by moisturisers, lotions and creams. "Generic advice has its limits," says Dr Trupti Agarwal, dermatology consultant at Sir H.N. Reliance Hospital in Mumbai. "People often add another product when the skin starts reacting, when what it may need first is fewer products and a diagnosis."

"We are seeing more patients in whom an itch is not a one-off episode but keeps returning as the trigger has not gone away"



—DR MONICA BAMBROO
Head of Dermatology & Cosmetology, Artemis Hospital, Gurugram

Apart from environmental irritants and cosmetics, the reasons may sometimes include more serious conditions like diabetes, thyroid, kidney or liver disorders, or lymphatic malignancy. Autoimmune conditions are another factor; psoriasis is one such disorder, whose thick, scaly patches can itch or burn. Age and hormonal changes bring